Atria Private Care

Standard Operating Procedures Binder

Oct 13, 2026

About this binder

This binder holds the 52 standard operating procedures (SOPs) every Atria Private Care staff member follows, written for private home care nursing in Ontario.

Item	Detail
Organization	Atria Private Care
Version	1.0
Approved by	Taegan, RN, Clinical Lead
Applies to	All Atria staff: RNs, RPNs, PSWs, companions, office and on-call staff
Review cycle	Every SOP yearly, or sooner after a serious incident or a change in law or best practice
Legal context	Ontario: Regulated Health Professions Act, Nursing Act and College of Nurses of Ontario (CNO) standards, Health Care Consent Act, PHIPA, Occupational Health and Safety Act

How each SOP is laid out: a lead sentence stating the rule, a scope table (who it applies to, what it covers, related SOPs, version), core principles, numbered procedure steps, and what to document. Key SOPs add a timeline table for reporting. Where an SOP and a client's care plan differ, follow the care plan and tell the RN. Where an SOP and the law differ, the law wins.

Staff acknowledgement: every staff member reads this binder at orientation and after each new version, then signs the acknowledgement form kept in their HR file.

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Part 1: RN and Administration

For the RN, clinical lead, owner and office staff: intake and care planning, consent, delegation, hiring, supervision, quality and records.

Section 1: Client intake and care planning

SOP 1: Client inquiry and intake

Every inquiry gets a response within 1 business day and a clear decision on whether Atria can safely meet the client's needs.

Item	Detail
Applies to	Office staff, intake coordinator, RN
Covers	Phone, email, web and referral inquiries for new clients
Related SOPs	2 Initial nursing assessment, 4 Service agreement
Version / approval	1.0 · Taegan, RN

Core principles

- **Safety before sales.** Accept only clients whose needs Atria staff can meet safely and within scope.
- **Be honest early.** If Atria is not the right fit, say so and help the family find who is.

Procedure

1. **Respond.** Reply to every inquiry within 1 business day.
2. **Collect the basics.** Client name, contact, address, substitute decision-maker (SDM), physician or NP, main care needs, funding (private pay, insurance, Ontario Health atHome top-up), and requested start date.
3. **Screen for safety.** Home access, pets, smoking, firearms, other people in the home, and infection risk.
4. **Decide fit.** If needs are beyond Atria's services or staff scope, refer to an appropriate provider.
5. **Book the assessment.** If suitable, book the initial nursing assessment (SOP 2).

Documentation: intake form in the client file, including the outcome of the inquiry.

SOP 2: Initial nursing assessment

A registered nurse assesses every client in their home before service starts.

Item	Detail
Applies to	RNs
Covers	All new clients, and returning clients after a break in service of 30 days or more

Item	Detail
Related SOPs	3 Care plan, 7 Consent, 31 Falls
Version / approval	1.0 · Taegan, RN

Core principles

- **Assess before care.** No staff member provides care before the RN has assessed the client (or within 24 hours for urgent starts).
- **The client's goals lead.** Record what matters to the client in their own words.

Procedure

1. **Get consent.** Confirm consent for the assessment and care (SOP 7).
2. **Assess the whole person.** Health history, diagnoses, medications and allergies, cognition, mobility and falls risk, skin, continence, nutrition and swallowing, pain, mood, home safety, and supports.
3. **Use standard tools.** Falls risk screen, Braden scale, pain scale, and others where they apply.
4. **Identify the team.** Physician or NP, pharmacy, SDM, family, and other providers.
5. **Set goals.** Agree on the client's goals for care.

Documentation: assessment form, signed and dated by the RN, within 24 hours of the visit.

SOP 3: Care plan development and review

Every client has a current written care plan, and staff follow it at every visit.

Item	Detail
Applies to	RNs (write and review); all care staff (follow)
Covers	All active clients
Related SOPs	2 Assessment, 23 Change in condition
Version / approval	1.0 · Taegan, RN

Core principles

- **One plan, followed by everyone.** Staff never change care on their own; they report changes to the RN.
- **A plan is only as good as its last review.** Update it whenever the client changes.

Procedure

1. **Write it.** The RN writes the care plan within 48 hours of the assessment, with the client and SDM.
2. **Include the essentials.** Goals, tasks per visit, transfer method and equipment, fall plan, medication support, diet, risks, emergency contacts, and what staff must report.
3. **Share it.** Staff get the care plan before their first visit; a copy stays in the home.
4. **Review it.** At least every 3 months, and after any fall, hospital stay, incident, or change in condition.

When	Who	Action
Within 48 hours of assessment	RN	Write care plan
Before first visit	Office	Send plan to assigned staff
Every 3 months, or after any change	RN	Review and update

Documentation: dated care plan versions, with the client's or SDM's acknowledgement.

SOP 4: Service agreement and fees

Clients know exactly what they are paying for before service starts.

Item	Detail
Applies to	Office staff, owner
Covers	All privately paid services
Related SOPs	1 Intake, 6 Discharge
Version / approval	1.0 · Taegan, RN

Core principles

- **No surprises.** Rates, minimums and cancellation terms are in writing before care starts.
- **Plain language.** Explain the agreement so the client understands it.

Procedure

1. **Prepare the agreement.** Services, schedule, hourly rates, minimum visit length, travel charges, holiday rates, cancellation policy, and billing frequency.
2. **Review it together.** Answer questions and offer time to consider it.
3. **Get it signed.** By the client or the person legally responsible for payment.
4. **Give notice of changes.** 30 days' written notice before any rate change.

Documentation: signed agreement in the client file.

SOP 5: Scheduling, late and missed visits

Clients receive every scheduled visit, on time, from a qualified staff member.

Item	Detail
Applies to	Office and scheduling staff, all care staff
Covers	All scheduled visits
Related SOPs	37 Working alone, 38 Business continuity
Version / approval	1.0 · Taegan, RN

Core principles

- **Consistency.** Aim for the same caregivers for each client.
- **A missed visit is a safety issue** for clients who live alone, need medication, or cannot transfer.

Procedure

1. **Match staff.** By skills, care needs, and client preference where possible.
2. **Running late.** Staff more than 15 minutes late call the office and the client.
3. **Cannot attend.** The office arranges a replacement and tells the client before the visit time.
4. **High-risk missed visit.** Escalate to the on-call RN immediately and arrange a welfare check.
5. **No unapproved swaps.** Staff do not swap shifts without office approval.

Documentation: late or missed visits logged, with the reason and follow-up.

SOP 6: Discharge and transfer of care

Service ends or moves to another provider safely, with no gap in care.

Item	Detail
Applies to	RN, office staff
Covers	Goals met, client request, hospital or long-term care admission, needs beyond scope, staff safety concerns
Related SOPs	9 Privacy, 48 Keys
Version / approval	1.0 · Taegan, RN

Core principles

- **No gap in care.** The next provider has what they need before Atria steps away.
- **Share only with consent.**

Procedure

1. **Give notice.** Written notice, 14 days where possible (except client request or immediate safety risk).
2. **Send a transfer summary.** With consent: diagnoses, medications, allergies, care needs, risks.
3. **Close out the home.** Return keys and client property; collect agency equipment.
4. **Hospital admission.** Put service on hold and confirm the plan with the family.

Documentation: discharge summary and the date service ended.

Section 2: Consent, rights and advance care planning

SOP 7: Consent and substitute decision-makers

Care is given only with valid consent from a capable client or their substitute decision-maker, as Ontario's Health Care Consent Act requires.

Item	Detail
Applies to	All care staff; RNs assess capacity

Item	Detail
Covers	Consent to assessment, treatment and personal care
Related SOPs	10 Advance care planning, 33 Emergencies
Version / approval	1.0 · Taegan, RN

Core principles

- **Consent is informed, voluntary and specific,** and can be withdrawn at any time.
- **A capable client may refuse,** even if staff disagree.

Procedure

1. **Explain.** What you plan to do, why, the risks, and the alternatives, in words the client understands.
2. **Assess capacity.** The RN assesses capacity for each treatment decision.
3. **If not capable, go to the SDM** in the order set out in the Act: court-appointed guardian, attorney for personal care, then family in the legislated order.
4. **Respect refusals.** Explain the risks, do not force care, and report the refusal to the RN.
5. **Emergencies.** If delay would cause serious harm and consent cannot be obtained, call 911.

Documentation: who gave consent, for what, and any refusal.

SOP 8: Client rights and complaints

Clients are treated with dignity and can raise concerns without fear, and every complaint gets a response within 2 business days.

Item	Detail
Applies to	All staff
Covers	Verbal and written complaints from clients, families and SDMs
Related SOPs	47 Incidents, 49 Quality improvement
Version / approval	1.0 · Taegan, RN

Core principles

- **Complaints are welcome.** They show what to fix.
- **No retaliation,** ever, against a client or family who complains.

Procedure

1. **Give the rights statement.** Every client receives the Atria Client Rights statement and complaint process at intake.
2. **Receive the complaint.** Thank the client, write it down, and tell the office the same day.
3. **Acknowledge and resolve.** Follow the timeline below.
4. **Close the loop.** Tell the client the outcome and what was changed. If unresolved, explain external options (e.g. CNO for nursing practice concerns).

When	Who	Action
Same day	Staff	Report complaint to office
Within 2 business days	Office	Acknowledge to client
Within 10 business days	Owner or RN	Resolve and respond

Documentation: complaint log, actions, and outcome.

SOP 10: Advance care planning and DNR

Staff know and respect each client's wishes about life-sustaining treatment, and can find the DNR Confirmation Form fast.

Item	Detail
Applies to	RNs (record wishes); all staff (follow)
Covers	Advance care plans, powers of attorney for personal care, DNR Confirmation Forms
Related SOPs	30 Palliative care, 33 Emergencies, 34 Unexpected death
Version / approval	1.0 · Taegan, RN

Core principles

- **No valid form, full response.** Without a DNR Confirmation Form in the home, staff call 911 and start CPR if trained.
- **Wishes change.** Review them often.

Procedure

1. **Ask at intake.** Does the client have an advance care plan, a power of attorney for personal care, or a DNR Confirmation Form?
2. **Record it.** Wishes and the SDM's name go in the care plan.
3. **Make the form findable.** Keep it where paramedics can find it (commonly on the fridge or with the client).
4. **Review.** At each care plan review and whenever the client's condition changes.

Documentation: advance care planning discussion and location of the DNR form.

Section 3: Medication oversight

SOP 21: Delegation and teaching of controlled acts

Unregulated staff perform a controlled act only after an RN has taught and authorized them for that specific client.

Item	Detail
Applies to	RNs (teach and authorize); PSWs (perform)
Covers	Tasks such as insulin injection and some feeding-tube care
Related SOPs	18 Administration, 28A Hypoglycemia, 28B Hyperglycemia, 44 Supervision
Version / approval	1.0 · Taegan, RN

Core principles

- **Client-specific.** Teaching for one client never transfers to another.
- **Stable and predictable only.** The RN authorizes only when the client and task are predictable.

Procedure

1. **Assess.** The RN confirms the client is stable and the task suits the CNO's guidance on authorizing mechanisms.
2. **Teach and observe.** The RN teaches the PSW and watches them do it correctly.
3. **Write it down.** Client-specific instructions in the care plan, including what to report.
4. **Recheck.** At least yearly or when the client's condition changes.
5. **Refuse if untaught.** Staff never do a task they have not been taught; they call the RN.

Documentation: teaching record signed by the RN and the PSW, in the client and staff files.

Section 4: Staff management

SOP 39: Hiring and credential checks

Atria hires only staff whose registration, credentials and background checks are verified before their first shift.

Item	Detail
Applies to	Owner, hiring managers
Covers	All new hires and yearly re-verification
Related SOPs	40 Orientation, 44 Supervision
Version / approval	1.0 · Taegan, RN

Core principles

- **Verify, don't assume.** Check every credential against its source.
- **No checks, no shifts.**

Procedure

1. **Nursing registration.** Check the CNO public register before hire and yearly (and any other province where the nurse will practise).
2. **Credentials.** Verify PSW certificates and other credentials against the originals.
3. **Vulnerable Sector Check.** Before hire, renewed at least every 3 years (or per contract).
4. **References.** At least 2, including a recent supervisor.

5. **Driving and life support.** Valid driver's licence and auto insurance for staff who drive for work; current BLS/CPR.

Documentation: copies and verification dates in the staff file.

SOP 40: Orientation and mandatory training

Every staff member completes orientation and required training before working alone with a client.

Item	Detail
Applies to	All staff
Covers	Orientation, shadow shifts, yearly refreshers, legislated training
Related SOPs	32 Transfers, 39 Hiring
Linked training	Move Safe — Client Handling for Home Care
Version / approval	1.0 · Taegan, RN

Core principles

- **Trained before alone.**
- **Training expires.** Track renewal dates.

Procedure

1. **Orientation** before the first shift: this binder, client rights, privacy, IPAC, emergencies, documentation, workplace violence and harassment.
2. **Move Safe module** before any transfers, and again when updated.
3. **Shadow shift.** At least 1 shift with an experienced staff member, or as the RN decides.
4. **Yearly refreshers.** IPAC and hand hygiene, safe handling, privacy, workplace violence, falls response, BLS/CPR as required.
5. **Legislated training.** Health and safety awareness (worker and supervisor), WHMIS, and AODA.

Documentation: training records with dates and expiry in each staff file.

SOP 44: Supervision and performance review

Every staff member gets a supervisory visit within 3 months of hire and at least yearly after.

Item	Detail
Applies to	RN, supervisors, all staff
Covers	Supervisory visits, performance reviews, discipline
Related SOPs	21 Delegation, 39 Hiring
Version / approval	1.0 · Taegan, RN

Core principles

- **Coach early.** Most concerns are fixed with support.
- **Some reports are mandatory** to the CNO.

Procedure

1. **Supervisory visit** within the first 3 months and at least yearly.
2. **Check skills in the home.** Hand hygiene, transfers, documentation, communication.
3. **Yearly review** with goals and training needs.
4. **Address concerns.** Coaching first; progressive discipline for serious or repeated concerns.
5. **Report to CNO** when required (e.g. termination for professional misconduct, incompetence or incapacity).

Documentation: supervisory visit forms and reviews in the staff file.

Section 5: Quality and records

SOP 49: Quality improvement and client feedback

Atria tracks quality monthly, reviews it quarterly, and sets 1 or 2 improvement goals each year.

Item	Detail
Applies to	Owner, RN, management
Covers	Client surveys, quality indicators, improvement plans
Related SOPs	8 Complaints, 47 Incidents
Version / approval	1.0 · Taegan, RN

Core principles

- **Measure what matters** to client safety and experience.
- **Share results** with staff.

Procedure

1. **Survey** clients and families at least yearly and after discharge.
2. **Track monthly:** falls, medication incidents, missed visits, complaints, skin injuries, staff injuries.
3. **Review quarterly** with the RN and management.
4. **Set goals.** 1 or 2 per year, with clear actions and owners.
5. **Share** results and changes with staff.

Documentation: quality reports and improvement plans.

SOP 50: Record retention and destruction

Client records are kept at least 10 years and destroyed securely, with a log.

Item	Detail
Applies to	Owner, office staff
Covers	Client, staff and WSIB records, paper and electronic
Related SOPs	9 Privacy, 46 Documentation
Version / approval	1.0 · Taegan, RN

Core principles

- **Keep long enough, destroy securely.**

Procedure

1. **Client records.** At least 10 years after the last entry, or until 10 years after the client turns 18, whichever is later (Atria policy; confirm with legal advice).
2. **Staff and WSIB records** as required by employment and WSIB rules.
3. **Store securely.** Locked cabinets for paper; password-protected, backed-up systems with access logs for electronic.
4. **Destroy securely.** Cross-cut shredding or a certified destruction service; wipe devices before disposal.
5. **Log it.** Date and method for every record destroyed.

Documentation: records destruction log.

Part 2: PSWs and Direct Care Staff

For PSWs, companions and nurses providing hands-on care: what to do in the client's home, every visit.

Section 6: Privacy, conduct and safeguarding

SOP 9: Privacy and confidentiality

Personal health information is collected, used and shared only as needed for care, as PHIPA requires.

Item	Detail
Applies to	All staff
Covers	Paper and electronic records, conversations, phones, photos, email and messaging
Related SOPs	46 Documentation, 50 Record retention
Version / approval	1.0 · Taegan, RN

Core principles

- **Need to know only.** Share with the client, SDM and care team, unless the client consents or the law requires it.
- **Report breaches fast.** Every minute counts.

Procedure

1. **Collect only what is needed** for care.
2. **Keep conversations private.** Never discuss clients in public, on social media, or with your own family.
3. **No personal devices for client photos.** Never photograph clients on personal devices.
4. **Use approved systems.** Only Atria-approved apps and devices; lock devices; no client details over personal messaging apps.
5. **Report breaches.** Lost phone, wrong email recipient, overheard conversation: tell the office immediately. The privacy officer notifies affected clients and, where required, the Information and Privacy Commissioner of Ontario.

Documentation: consent to share, and any breach report.

SOP 11: Suspected abuse or neglect

Any staff member who suspects abuse or neglect reports it to the RN or supervisor the same day, and calls 911 if the client is in immediate danger.

Item	Detail
Applies to	All staff
Covers	Physical, emotional, sexual and financial abuse, and neglect, by anyone
Related SOPs	45 Client money, 47 Incidents
Version / approval	1.0 · Taegan, RN

Core principles

- **Report, don't investigate.** Never confront the suspected person yourself.
- **Some reports are the law.** Child protection and sexual abuse by a regulated professional must be reported.

Procedure

1. **Know the signs.** Unexplained injuries, fear of a person, sudden money changes, missing belongings, poor hygiene or lack of food, being kept isolated.
2. **Immediate danger.** Call 911.
3. **Report.** Tell the RN or supervisor the same day.
4. **Child protection.** If a child may need protection, report directly to a children's aid society (Child, Youth and Family Services Act).
5. **Sexual abuse by a regulated professional.** Regulated staff report as required by the Regulated Health Professions Act.
6. **Support.** With the capable client's consent, connect them with help (e.g. police, the Seniors Safety Line).

Documentation: facts only: what you saw and heard, the date and time, and who you told.

SOP 41: Code of conduct and professional boundaries

Staff keep every relationship with clients and families professional.

Item	Detail
Applies to	All staff
Covers	Conduct, communication, social media, personal relationships, impairment
Related SOPs	45 Client money and gifts
Version / approval	1.0 · Taegan, RN

Core principles

- **Friendly, not friends.**
- **Never work impaired.**

Procedure

1. **Show up professionally.** On time, in uniform, ID badge visible; respectful to everyone.
2. **Keep personal life separate.** No sharing personal problems, phone numbers, or social media; no friend requests.
3. **No personal business.** No selling, borrowing, lending, or buying from clients.
4. **Fit for duty.** No smoking, vaping, alcohol or cannabis on shift; no personal phone use during care except emergencies.

5. **No romantic or sexual relationships** with clients, ever. Report boundary concerns to the supervisor.

Documentation: signed code of conduct in the staff file.

SOP 42: Workplace violence and harassment

Atria protects staff from violence and harassment and investigates every report, as Ontario's OHSA requires.

Item	Detail
Applies to	All staff, clients, families
Covers	Physical violence, threats, verbal abuse, sexual harassment
Related SOPs	29 Responsive behaviours, 37 Working alone
Version / approval	1.0 · Taegan, RN

Core principles

- **Staff are told about known risks** before the visit.
- **No reprisal** for reporting.

Procedure

1. **Review policies yearly** and assess risk for each client.
2. **Share risks.** History of violence, aggressive pets, weapons in the home go in the care plan.
3. **If violence happens or is threatened,** leave to a safe place, call 911 if needed, then the supervisor.
4. **Report everything,** including verbal abuse and sexual harassment, from anyone.
5. **Investigate and act.** Change the care plan (e.g. two staff, alternate staff) as needed.

Documentation: incident report and investigation record.

SOP 43: Staff injury and WSIB reporting

Injured staff get care first, and Atria reports to WSIB within 3 business days when required.

Item	Detail
Applies to	All staff, owner
Covers	Work injuries, needlesticks, occupational illness
Related SOPs	16 Sharps, 32 Transfers
Version / approval	1.0 · Taegan, RN

Core principles

- **Care first, forms second.**
- **Report every injury,** even minor ones.

Procedure

1. **Get care.** First aid or medical care right away, then tell the supervisor.
2. **Transport.** Atria provides transportation to medical care on the day of injury if needed.
3. **Report.** Follow the timeline below.
4. **Return to work.** Modified duties and a return-to-work plan.

When	Who	Action
Immediately	Staff	Get care, tell supervisor
Immediately (critical injury)	Owner	Notify Ministry of Labour
Within 3 business days	Owner	WSIB Form 7 if health care beyond first aid or lost time

Documentation: incident report, WSIB forms, and return-to-work plan.

SOP 45: Client money, gifts and property

Staff do not accept money or gifts from clients and handle client money only as the care plan allows, with receipts.

Item	Detail
Applies to	All staff

Item	Detail
Covers	Gifts, cash, bank cards, errands, wills and powers of attorney
Related SOPs	11 Abuse, 41 Code of conduct
Version / approval	1.0 · Taegan, RN

Core principles

- **Protects clients and staff.** A clear paper trail prevents allegations.

Procedure

1. **No gifts.** No money, loans, or inheritances; a small token only with supervisor approval.
2. **No banking access.** No bank cards, PINs, chequebooks or cash unless the care plan allows it.
3. **Errands.** Written log with receipts and change returned, signed by the client.
4. **No personal use** of the client's phone, car, or belongings.
5. **Report** missing items, or any request to change a will or power of attorney.

Documentation: errand log and any gift approval.

Section 7: Infection prevention and control

SOP 12: Hand hygiene

Staff clean their hands at the 4 moments of every visit, the single most effective way to stop infection spreading.

Item	Detail
Applies to	All care staff
Covers	Every client visit
Related SOPs	13 Routine practices and PPE
Version / approval	1.0 · Taegan, RN

Core principles

- **Hand rub first,** soap and water when hands are visibly soiled.
- **Gloves never replace hand hygiene.**

Procedure

1. **Clean at the 4 moments.** Before contact with the client or their environment, before a clean or aseptic procedure, after body fluid exposure risk, and after contact with the client or their environment.
2. **Hand rub.** Alcohol-based rub (60–90% alcohol) for 15–20 seconds.
3. **Soap and water.** At least 15 seconds when hands are visibly soiled, and after caring for a client with diarrhea (e.g. C. difficile or norovirus).
4. **Keep hands ready.** Short natural nails; no artificial nails or hand jewellery other than a plain band.
5. **Carry it.** Hand rub goes to every visit.

Documentation: hand hygiene audits by the supervisor, at least twice a year per staff member.

SOP 13: Routine practices and PPE

Staff treat all blood and body fluids as potentially infectious and choose PPE by a quick risk assessment before every task.

Item	Detail
Applies to	All care staff
Covers	Gloves, gowns, masks, eye protection, respirators
Related SOPs	12 Hand hygiene, 14 Additional precautions
Version / approval	1.0 · Taegan, RN

Core principles

- **Assess, then protect.** Ask: what could I be exposed to?
- **Order matters.** Putting on and taking off PPE in the right order prevents self-contamination.

Procedure

1. **Point-of-care risk assessment** before every task.

2. **Gloves** for contact with body fluids, broken skin or mucous membranes; change between tasks and clean hands after removing.
3. **Gown** if clothing could be splashed; **mask and eye protection** if the face could be splashed.
4. **Put on:** gown, mask, eye protection, gloves. **Take off:** gloves, gown, clean hands, eye protection, mask, clean hands.
5. **Carry a PPE kit** to every visit: gloves, gowns, medical masks, N95 respirator if fit-tested, eye protection.

Documentation: PPE training and fit-testing in the staff file.

SOP 14: Additional precautions

Staff add the right protection, in addition to routine practices, when a client has or may have a transmissible infection.

Item	Detail
Applies to	All care staff; RN sets precautions
Covers	Contact, droplet and airborne precautions
Related SOPs	13 PPE, 15 Illness screening
Version / approval	1.0 · Taegan, RN

Core principles

- **Staff know before they arrive.** Precautions are in the care plan and shared before the visit.
- **Follow current Public Health Ontario guidance.**

Precaution	Examples	What to wear
Contact	MRSA, C. difficile, scabies	Gloves and gown for all direct care
Droplet and contact	Influenza, RSV	Medical mask, eye protection, gown and gloves within 2 metres
Airborne	Tuberculosis, measles, chickenpox	Fit-tested N95; only immune staff for measles or chickenpox

Procedure

1. **Set precautions.** The RN adds them to the care plan and tells assigned staff.
2. **Keep equipment in the home** where possible.
3. **Plan the day.** Make the client on precautions the last visit of the day when practical.
4. **Lift precautions** only when the RN says so.

Documentation: precautions in the care plan, and staff told before their visit.

SOP 15: Illness screening and outbreaks

Sick staff stay away from clients, and new symptoms in clients are caught and reported early.

Item	Detail
Applies to	All staff
Covers	Staff self-screening, client symptom screening, reportable diseases
Related SOPs	14 Additional precautions
Version / approval	1.0 · Taegan, RN

Core principles

- **Don't work sick.** One sick worker can infect many clients.
- **Vaccination protects everyone.**

Procedure

1. **Self-screen** before each shift for fever, cough, sore throat, vomiting or diarrhea.
2. **Stay home if sick.** Call the office; return per current public health guidance (e.g. 48 hours after vomiting or diarrhea stops).
3. **Screen the household.** Before each visit, ask about new symptoms; if present, use the right precautions (SOP 14) and tell the RN.
4. **Report** reportable diseases to the local public health unit as required.
5. **Encourage vaccination.** Annual influenza and up-to-date COVID-19 vaccination for staff and clients.

Documentation: staff illness absences, and client symptoms reported to the RN.

SOP 16: Sharps and needlestick injuries

Staff prevent injuries from needles and lancets and get medical assessment within 2 hours of any exposure.

Item	Detail
Applies to	All staff who handle sharps
Covers	Needles, lancets, sharps containers, needlestick and splash exposures
Related SOPs	43 Staff injury and WSIB
Version / approval	1.0 · Taegan, RN

Core principles

- **Never recap a used needle.**
- **Exposure is urgent.** Treatment works best when started early.

Procedure

1. **Use safety-engineered** needles and lancets.
2. **Dispose at the point of use** into an approved sharps container; close it when three-quarters full.
3. **Return full containers** to a pharmacy or approved take-back program.
4. **If stuck or splashed:** let the wound bleed freely, wash with soap and water (flush eyes or mouth with water), then call the supervisor.
5. **Get assessed.** Go to an emergency department within 2 hours for possible post-exposure treatment.

Documentation: incident report, and WSIB report if required (SOP 43).

SOP 17: Cleaning equipment and waste

Shared equipment is disinfected after every client, and waste and laundry are handled safely.

Item	Detail
Applies to	All care staff

Item	Detail
Covers	Reusable equipment, soiled waste, linen, client equipment in the home
Related SOPs	13 PPE
Version / approval	1.0 · Taegan, RN

Core principles

- **Clean between every client.**
- **Contact time matters.** A wipe only disinfects if the surface stays wet for the product's contact time.

Procedure

1. **Disinfect equipment** (thermometers, BP cuffs, glucometers, stethoscopes) after each client with a hospital-grade wipe.
2. **Separate clean and dirty.** Keep a clean bag and a dirty bag in your visit kit.
3. **Bag waste.** Soiled dressings, incontinence products and gloves go in a tied bag in the household garbage.
4. **Linen.** Handle soiled linen with gloves and wash in hot water separately.
5. **Client equipment.** Clean commodes, raised toilet seats and bath seats as the care plan directs.

Documentation: equipment cleaning as part of the visit note.

Section 8: Medication management

SOP 18: Medication administration and assistance

Clients get the right medication safely, from staff working only within their role.

Item	Detail
Applies to	Nurses (administer); PSWs and companions (assist, or as taught under SOP 21)
Covers	All prescription and non-prescription medications
Related SOPs	19 Errors, 20 Storage, 21 Delegation, 22 Allergies

Item	Detail
Version / approval	1.0 · Taegan, RN

Core principles

- **Stay in your role.** PSWs only do what the care plan says or what an RN has taught them.
- **When something looks different, stop.** Never guess with medications.

Procedure

1. **Know your role.** Nurses administer within scope and CNO standards, with a current order where required. PSWs and companions assist only as written in the care plan (e.g. reminding, opening a blister pack the client takes themselves).
2. **Check the rights.** Right client, medication, dose, time, route, reason, and documentation. Check allergies.
3. **Use blister packs** where possible. Never pour medications ahead of time for later.
4. **Refusals and surprises.** If the client refuses, vomits, or a dose looks different, do not force or substitute; report to the RN.
5. **Keep the list current.** The RN reconciles the medication list after every hospital stay or prescription change.

Documentation: each dose given or assisted on the medication record, at the time it happens.

SOP 19: Medication errors and near misses

Any medication error is reported to the RN immediately, and the client is checked for harm first.

Item	Detail
Applies to	All staff
Covers	Wrong client, drug, dose, time or route; missed or doubled doses; near misses
Related SOPs	18 Administration, 47 Incidents
Version / approval	1.0 · Taegan, RN

Core principles

- **Client first, report second, paperwork third.**
- **Just culture.** Reporting is expected, not punished; near misses show where the system needs fixing.

Procedure

1. **Check the client.** Signs of harm (drowsiness, breathing changes, chest pain, allergic reaction): call 911.
2. **Call the RN** or on-call nurse right away; the nurse contacts the pharmacist or prescriber.
3. **Disclose.** Tell the client or SDM what happened.
4. **Report near misses** too.

When	Who	Action
Immediately	Staff	Check client, call RN (911 if harm)
Same shift	Staff	Incident report
Within 1 business day	RN	Review and follow-up

Documentation: incident report (SOP 47).

SOP 20: Medication storage, disposal and controlled substances

Client medications stay safely stored in the client's home and are disposed of through a pharmacy, to prevent loss, misuse and poisoning.

Item	Detail
Applies to	All care staff; RN sets up counts
Covers	Storage, refrigeration, opioids and other controlled substances, disposal
Related SOPs	18 Administration, 51 and 52 Travel
Version / approval	1.0 · Taegan, RN

Core principles

- **Medications belong to the client** and stay in their home.

- **Missing medication is reported the same day.**

Procedure

1. **Store safely.** Original or pharmacy packaging, away from heat, moisture, children and pets.
2. **Refrigerate** medications that need it (e.g. insulin not in use) at 2–8°C.
3. **Count controlled substances.** With consent, the RN sets up a count sheet; staff count and sign as the care plan directs.
4. **Dispose through a pharmacy.** Never flush or throw out medications.
5. **Never take medications home** or store them in your car, except as set out in SOPs 51 and 52.

Documentation: counts, discrepancies, and medications returned.

SOP 22: Allergies and anaphylaxis

Allergies are recorded for every client, and suspected anaphylaxis is treated immediately with epinephrine and a 911 call.

Item	Detail
Applies to	All care staff
Covers	Medication, food and latex allergies; anaphylaxis
Related SOPs	18 Administration, 33 Emergencies
Version / approval	1.0 · Taegan, RN

Core principles

- **Epinephrine first, 911 second.** Don't wait for symptoms to worsen.
- **Non-latex gloves for everyone.**

Procedure

1. **Record allergies** and the reaction in the care plan and medication list.
2. **Know the signs.** Hives, swelling of the face or throat, trouble breathing, wheezing, vomiting, dizziness or collapse.
3. **Give epinephrine.** The client's prescribed auto-injector into the outer thigh.
4. **Call 911.** Lay the client flat (or sitting if breathing is hard).

5. **Second dose.** After 5 minutes if there is no improvement and a second injector is available. Stay until paramedics take over.

Documentation: time of epinephrine, the client's response, and the incident report.

Section 9: Clinical care

SOP 23: Vital signs and change in condition

Any change in a client's condition is reported to the RN before staff leave the home, and emergencies go to 911 first.

Item	Detail
Applies to	All care staff
Covers	Vital signs, early warning signs, escalation
Related SOPs	3 Care plan, 33 Emergencies
Version / approval	1.0 · Taegan, RN

Core principles

- **Know their normal.** Small changes are early warnings.
- **"Just not themselves" counts.** Report it.

Procedure

1. **Measure.** Take vital signs as the care plan directs, and any time the client seems unwell.
2. **Watch for change.** New confusion, drowsiness, breathing harder, new pain, not eating or drinking, less urine, fever, a fall.
3. **Call 911** for the red flags below.
4. **Call the RN** for other changes, before leaving the home. Use SBAR: situation, background, assessment, recommendation.
5. **Follow the RN's direction** (physician or NP contact, extra visits, or emergency department).

Call 911 for
Not breathing normally or unresponsive
Chest pain
Signs of stroke: face drooping, arm weakness, slurred speech
Severe bleeding
Seizure

Documentation: the observation, the time, who you told, and what they advised.

SOP 24: Wound care and skin integrity

Nurses treat wounds per the order and wound care plan, and all staff work to prevent skin tears.

Item	Detail
Applies to	Nurses (wound care); all care staff (prevention, first aid for skin tears)
Covers	Wounds, skin tears, signs of infection
Related SOPs	25 Pressure injury, 9 Privacy (photos)
Version / approval	1.0 · Taegan, RN

Core principles

- **Prevention beats treatment.** Most skin tears come from pulling, bumping or dry skin.
- **Measure to know if it's healing.**

Procedure

1. **Treat to order.** Nurses assess, measure and treat per the physician or NP order.
2. **Reassess weekly.** Photograph only with consent and on an Atria-approved device.
3. **Prevent skin tears.** Moisturize twice daily, pad bed rails and wheelchair parts, use a slide sheet, and never pull on arms.

4. **If a skin tear happens.** Control bleeding, gently clean, lay the flap back in place, cover with a non-adherent dressing, mark the flap direction, and tell the nurse.
5. **Report infection signs.** Increasing redness, warmth, swelling, pus, odour, or fever.

Documentation: wound assessment and treatment in the nursing record each visit.

SOP 25: Pressure injury prevention

At-risk clients are repositioned and checked every visit so their skin stays intact.

Item	Detail
Applies to	All care staff; RN assesses risk
Covers	Clients who spend long periods in bed or a chair
Related SOPs	24 Wound care, 26 Continence
Version / approval	1.0 · Taegan, RN

Core principles

- **Pressure, moisture and friction** cause most injuries.
- **Redness that does not fade** is an early pressure injury.

Procedure

1. **Assess risk.** The RN uses the Braden scale at intake and with every change in condition.
2. **Reposition.** At least every 2 hours in bed; help shift weight in a chair every hour (or as the care plan directs).
3. **Float heels.** Pillow under the calves; use pressure-relieving cushions and mattresses as prescribed.
4. **Keep skin clean and dry.** Change incontinence products promptly.
5. **Check bony areas** (heels, sacrum, hips, elbows, back of head) every visit and report redness that does not fade.

Documentation: skin checks, repositioning, and any new redness or open areas.

SOP 26: Continence and catheter care

Continence care protects the client's dignity and skin, and catheter care prevents infection.

Item	Detail
Applies to	All care staff; nurses insert and change catheters
Covers	Toileting, incontinence products, urinary catheters
Related SOPs	21 Delegation, 25 Pressure injury
Version / approval	1.0 · Taegan, RN

Core principles

- **Dignity first.** Privacy, choice, and a calm manner.
- **A closed, downhill system** prevents catheter infections.

Procedure

1. **Toileting.** Offer on the schedule in the care plan; respond to requests promptly.
2. **Hygiene.** Clean front to back; barrier cream as directed; check skin.
3. **Catheter care.** Bag below the bladder and off the floor, no kinks, empty when two-thirds full, clean the site daily.
4. **Insertion.** Only nurses (or staff taught under SOP 21) insert or change catheters.
5. **Report.** No urine for 4 hours or more, cloudy or bloody urine, strong smell, fever, new confusion, or leaking.

Documentation: output where ordered, and anything unusual.

SOP 26A: Showering clients

Clients shower seated, with the water tested before it touches them and a staff member within arm's reach the whole time.

Item	Detail
Applies to	All care staff
Covers	Showers in the client's home, assisted or supervised

Item	Detail
Does not cover	Tub baths with a bath lift (follow the care plan and the lift's instructions)
Related SOPs	24 Skin integrity, 26 Continence, 31 Falls, 32 Transfers
Version / approval	1.0 · Taegan, RN

Core principles

- **The bathroom is the highest-risk room for falls.** Wet floors, low seats and hot water add up.
- **Never leave the client alone in the shower,** not even to get a towel.
- **Test the water every time.** Older skin burns faster and many clients can't feel heat well.
- **Dignity and choice.** Cover what you aren't washing, and let the client do as much as they safely can.

Before the shower

- [ ] Check the care plan: transfer method, shower equipment, skin precautions, wounds, catheter, and how often the client showers.
- [ ] Check the client today: any dizziness, new pain, shortness of breath, recent fall, or feeling unwell? If yes, offer a sponge bath and tell the RN.
- [ ] Offer the toilet first.
- [ ] Gather everything before you start: towels, washcloths, soap, shampoo, clean clothes, moisturizer, incontinence product, gloves.
- [ ] Set up the bathroom: non-slip mat in and outside the shower, shower chair or transfer bench locked in place, hand-held shower head within reach, floor dry and clear.
- [ ] Warm the room and close the door or window to stop drafts.
- [ ] Cover wounds and dressings with a waterproof cover as the nurse directs.

Procedure

1. **Explain and get consent.** Tell the client what will happen, step by step. If they say no, respect it, offer another time or a sponge bath, and report it.
2. **Test the water.** Run it before the client gets in. Aim for warm, about 37–40°C. Check with a bath thermometer, or on the inside of your wrist. Never above 43°C.

3. **Transfer to the seat.** Use the method in the care plan. The client sits for the whole shower. Use grab bars, never towel bars or the shower curtain.
4. **Let the client test it too,** if they can feel temperature. Start the water at their feet and move up.
5. **Wash clean to dirty.** Face (water only, no soap near eyes), hair, arms, chest, back, legs, feet, then the groin and bottom last, front to back. Use a long-handled sponge for areas the client can't reach.
6. **Keep them warm and covered.** Drape a towel over parts not being washed.
7. **Check the skin** as you wash: redness, bruises, skin tears, rashes, swelling, and between toes and skin folds.
8. **Turn off the water before the client moves.** Wrap them in a towel while seated.
9. **Dry thoroughly,** patting, not rubbing. Dry between toes and in skin folds. Moisturize dry skin, but not between toes.
10. **Dress while seated** where possible, starting with the lower body, and put on non-slip footwear before standing.
11. **Transfer out** using the care plan method. Take your time; the floor may still be wet.
12. **Clean up.** Wipe the shower chair, dry the floor, put dirty linen in the laundry, and clean your hands.

Stop the shower if

What you see	What to do
Dizziness, fainting, pale or clammy skin	Turn off water, keep client seated, wrap warmly; call RN; 911 if they don't improve or lose consciousness
Chest pain or shortness of breath	Turn off water, keep seated, call 911
Client slips or falls	Follow SOP 31; do not lift them off the floor
Distress, fear or resisting care	Stop, cover them, speak calmly; try a towel or sponge bath instead and report it
Water too hot or skin reddens	Turn off hot water immediately, cool the area with cool running water for 20 minutes, report to RN

Special situations

- **Dementia.** Showers can be frightening. Keep the routine the same, aim a hand-held shower low, wash hair last or on a separate day, and consider a towel bath if showers cause distress.
- **Catheter.** Keep the bag below the bladder and clean the catheter site with soap and water.
- **Oxygen.** Follow the care plan; keep tubing away from the water and the concentrator out of the bathroom.
- **Nail care.** Do not cut toenails for clients with diabetes or poor circulation; report long nails to the RN.
- **Electrical items.** No hair dryers, razors or heaters near water.

When	Who	Action
Same visit	Staff	Report new skin changes, refusals, or any stop event to the RN
Before end of shift	Staff	Incident report for a fall, burn or injury
At care plan review	RN	Reassess shower method and equipment

Documentation: shower given (or refused and what was offered instead), skin findings, the client's tolerance, and anything reported.

SOP 27: Nutrition, hydration and swallowing

Clients eat and drink safely, in the texture their care plan sets, to prevent choking, dehydration and malnutrition.

Item	Detail
Applies to	All care staff
Covers	Meals, fluids, modified textures (IDDSI), swallowing concerns
Related SOPs	23 Change in condition, 33 Emergencies
Version / approval	1.0 · Taegan, RN

Core principles

- **Texture is a safety order.** Never give a different texture than the care plan says.
- **Upright for eating.**

Procedure

1. **Follow the texture** in the care plan exactly (e.g. minced, puréed, mildly thick fluids, per IDDSI levels).
2. **Position.** Fully upright for meals and for 30 minutes after.
3. **Offer fluids** at every visit unless the client is on a fluid restriction.
4. **Report.** Coughing or choking with eating or drinking, a wet or gurgly voice, weight loss, poor intake, or mouth pain.
5. **Choking.** If the client cannot cough, speak or breathe: call 911 and give abdominal or chest thrusts as trained.

Documentation: intake when ordered, and any swallowing concern.

SOP 28A: Hypoglycemia (low blood glucose)

A blood glucose below 4.0 mmol/L is treated right away with 15 g of fast-acting sugar, rechecked in 15 minutes, and reported to the RN; a client who cannot swallow safely gets nothing by mouth and a 911 call.

Item	Detail
Applies to	All care staff; PSWs check glucose or give insulin only as taught under SOP 21
Covers	Clients with diabetes on insulin or medications that can cause lows (e.g. gliclazide, glyburide)
Related SOPs	21 Delegation, 23 Change in condition, 28B Hyperglycemia, 33 Emergencies
Version / approval	1.0 · Taegan, RN

Core principles

- **Treat first, then report.** A low can become an emergency within minutes.
- **If in doubt, treat it as a low.** If there is no meter and the client has symptoms, give sugar.
- **Nothing by mouth** for a client who is drowsy, confused, or cannot swallow safely.

- **Older adults may have few warning signs.** Confusion or unusual behaviour can be the only clue.

Know the signs

Early (mild to moderate)	Severe
Shaking, sweating, pale skin, fast heartbeat, hunger, anxiety, headache, blurred vision, irritability	Confusion, unusual behaviour, slurred speech, drowsiness, unable to follow directions, seizure, unresponsive

At night, watch for nightmares, damp sheets or pyjamas, or morning headache.

Procedure: client is awake and can swallow

1. **Check the blood glucose.** Wash and dry the client's hands first. If there is no meter, go straight to step 2.
2. **Give 15 g of fast-acting sugar.** One of: 4 glucose tablets (4 g each), 150 mL (⅔ cup) of juice or regular pop, 1 tablespoon of sugar or honey, or 6 Life Savers. Glucose tablets are best. If the client takes acarbose, use glucose tablets, milk or honey only.
3. **Wait 15 minutes** with the client seated or lying down. Do not leave them alone. Do not give chocolate or cookies first; fat slows sugar absorption.
4. **Recheck.** If still below 4.0 mmol/L, or symptoms continue, give another 15 g and wait 15 minutes again.
5. **If still low after 2 treatments,** or the client is getting worse, call 911.
6. **Once above 4.0,** give a snack with 15 g carbohydrate and protein (e.g. half a sandwich, or crackers and cheese) if the next meal is more than 1 hour away.
7. **Call the RN before leaving the home.** The RN decides on follow-up checks, especially for clients on long-acting insulin or sulfonylureas, where lows can return for hours.

Procedure: client is drowsy, confused, cannot swallow, or is unresponsive

1. **Call 911.**
2. **Give nothing by mouth.** Do not put food, juice or glucose gel in the mouth of a client who cannot swallow safely.
3. **Give glucagon** if it is prescribed and you are trained and authorized (nasal or injection, per the care plan).
4. **Position.** If unresponsive and breathing, roll the client onto their side.
5. **If not breathing normally,** start CPR if trained and there is no DNR Confirmation Form.

6. **Stay** until paramedics take over, then call the supervisor.

When	Who	Action
Immediately	Staff	Treat; call 911 if severe
Before leaving the home	Staff	Call RN or on-call nurse
Same shift	Staff	Incident report for a severe low or 911 call
Within 1 business day	RN	Review cause (missed meal, dose, activity, illness) and update care plan

Documentation: glucose readings with times, what was given and how much, the client's response, and who was notified.

SOP 28B: Hyperglycemia (high blood glucose)

A reading above the client's care plan limit is reported to the RN the same visit, and high glucose with vomiting, deep fast breathing, or drowsiness is a 911 call.

Item	Detail
Applies to	All care staff; PSWs check glucose or give insulin only as taught under SOP 21
Covers	Clients with type 1 or type 2 diabetes; high glucose from illness, missed medication, steroids or other causes
Related SOPs	21 Delegation, 23 Change in condition, 28A Hypoglycemia, 33 Emergencies
Version / approval	1.0 · Taegan, RN

Core principles

- **The care plan sets the client's limits.** The RN writes each client's target range and the reading that must be reported.
- **Staff never stop, skip or change diabetes medications** on their own. Only the prescriber or RN does, using the client's sick-day plan.
- **Illness is the most common trigger.** A client who is unwell needs closer checks.

- **Look for the cause:** missed medication, infection, steroids, more food, less activity, stress.

Know the signs

Report to RN (same visit)	Call 911
Reading above the care plan limit; thirst; passing more urine; dry mouth; tiredness; blurred vision; headache	Nausea or vomiting; stomach pain; fruity-smelling breath; deep or fast breathing; drowsiness or new confusion; signs of severe dehydration; meter reads "HI" with the client unwell

The 911 signs can mean diabetic ketoacidosis (DKA), most common in type 1, or hyperosmolar hyperglycemic state (HHS), most common in older adults with type 2. Both are life-threatening.

Procedure

1. **Recheck the reading.** Wash and dry the client's hands and test again. Sugar on the fingers gives a false high.
2. **Check for 911 signs** (table above). If any are present, call 911, then the supervisor.
3. **If no 911 signs,** call the RN or on-call nurse before leaving the home. Report the reading, symptoms, what the client ate, medications taken or missed, and any signs of illness.
4. **Check ketones** if the client has type 1 diabetes and the reading is above 14.0 mmol/L, or as the care plan directs, and report the result.
5. **Encourage sugar-free fluids** (water is best) if the client can swallow and has no fluid restriction.
6. **Follow the RN's direction** on rechecks, medications and the sick-day plan.
7. **If the client takes an SGLT2 inhibitor** (e.g. empagliflozin, dapagliflozin, canagliflozin) and has nausea, vomiting, stomach pain or feels very unwell, report to the RN urgently **even if the glucose reading is normal.** These medications can cause ketoacidosis without high glucose.

When	Who	Action
Immediately	Staff	Recheck; call 911 if danger signs
Before leaving the home	Staff	Call RN or on-call nurse

When	Who	Action
As directed	Staff	Recheck glucose and ketones; report
Within 1 business day	RN	Review cause, contact prescriber if needed, update care plan

Documentation: glucose and ketone readings with times, symptoms, fluids and food taken, medications taken or missed, and who was notified.

SOP 29: Dementia and responsive behaviours

Staff give calm, person-centred care and treat responsive behaviours as a sign of an unmet need.

Item	Detail
Applies to	All care staff
Covers	Clients living with dementia or cognitive impairment
Related SOPs	36 Missing client, 42 Workplace violence
Version / approval	1.0 · Taegan, RN

Core principles

- **Behaviour is communication.** Look for pain, hunger, toileting, fear, boredom or over-stimulation.
- **Never argue, restrain or force care.**

Procedure

1. **Know the person.** History, routines, preferences, and what calms them, in the care plan.
2. **Approach well.** From the front, introduce yourself every time, speak slowly, one step at a time.
3. **Find the need** behind resisting care, calling out, pacing, or striking out.
4. **Step back.** If the client is distressed, stop, keep yourself safe, and try again later or another way.

5. **Report change.** New or worsening behaviour may be delirium from infection or medication.

Documentation: the behaviour, what came before it, and what helped (an ABC record when requested).

SOP 30: Palliative care and expected death at home

Clients at the end of life get comfort-focused care, and an expected death at home is handled calmly without a 911 call.

Item	Detail
Applies to	All care staff; RN confirms death
Covers	Palliative care and expected death with a plan in place
Related SOPs	10 Advance care planning, 34 Unexpected death
Version / approval	1.0 · Taegan, RN

Core principles

- **Comfort and dignity** guide every task.
- **Have the plan before it's needed.** Expected-death plan and DNR form in place.

Procedure

1. **Confirm the plan.** Goals of care, DNR Confirmation Form, and expected-death plan with client, family, and physician or NP.
2. **Provide comfort.** Pain and symptom control as ordered, mouth care, repositioning, emotional support.
3. **Report distress** (uncontrolled pain, breathlessness, agitation) to the RN right away.
4. **When death occurs as expected,** do not call 911. Call the RN, who confirms death per CNO standards and the plan, and notifies the physician or NP and funeral home.
5. **Support the family** and return keys and agency items afterward.

Documentation: time of death, who was present, and who was notified.

Section 10: Safety and emergencies

SOP 31: Falls prevention and response

Staff prevent falls at every visit and respond to a fall without causing more harm; the full procedure is in Standard Operating Procedure: Client Falls Response.

Item	Detail
Applies to	All care staff; RN completes risk assessments
Covers	Falls inside and outside the home
Related SOPs	32 Transfers, 33 Emergencies, 47 Incidents
Linked training	Move Safe — Client Handling for Home Care
Version / approval	1.0 · Taegan, RN

Core principles

- **Stop, don't rush.** A fast lift off the floor causes more harm than a few minutes of assessment.
- **Never lift a client off the floor manually.** If they cannot get up on their own with coaching, call 911.
- **Every fall is reported,** including those with no visible injury.

Procedure

1. **Assess risk.** The RN completes a falls risk assessment at intake and after any fall, and writes a fall plan in the care plan.
2. **Prevent at every visit.** Clear paths; check lighting, footwear, glasses and the mobility aid; keep the call device within reach.
3. **If a client falls.** Make the scene safe, check responsiveness and breathing, keep them still, and check for red flags.
4. **Getting up.** Only if there are no red flags and the client can do it themselves with coaching. Otherwise call 911.
5. **Report.** Call the supervisor before leaving; complete the incident report the same shift.

Documentation: incident report and post-fall monitoring.

SOP 32: Safe client handling and transfers

Staff move clients only by the method and equipment in the care plan, and never manually lift more than 35 lb of a client's weight.

Item	Detail
Applies to	All care staff
Covers	Transfers, repositioning, mobility assistance, mechanical lifts
Related SOPs	31 Falls, 43 Staff injury
Linked training	Move Safe — Client Handling for Home Care
Version / approval	1.0 · Taegan, RN

Core principles

- **Never improvise a transfer.**
- **Good body mechanics alone don't prevent injury.** Equipment does.
- **Staff may refuse an unsafe transfer** and report it.

Procedure

1. **Follow the care plan** for the transfer method and equipment.
2. **Stop and check.** The client's condition today, equipment working, brakes on, path clear, footwear on.
3. **Respect the 35 lb (16 kg) limit.** Use equipment instead.
4. **Mechanical lifts.** Only if trained on that model; check sling size and condition; use the number of staff the care plan requires.
5. **Stop and get help** if the client cannot bear weight as expected or becomes unwell.

Documentation: any change in the client's mobility.

SOP 33: Medical emergencies and 911

In a life-threatening emergency, staff call 911 first, then the supervisor, and stay with the client until paramedics take over.

Item	Detail
Applies to	All staff
Covers	Any medical emergency during a visit or outing
Related SOPs	10 DNR, 23 Change in condition, 34 Unexpected death
Version / approval	1.0 · Taegan, RN

Core principles

- **911 first.** Paperwork and phone calls can wait.
- **Never drive a client in emergency distress** to hospital yourself.

Procedure

1. **Call 911.** Give the exact address, entrance, and how to get in.
2. **Help paramedics in.** Unlock the door and turn on outside lights.
3. **CPR and AED** if trained and there is no DNR Confirmation Form.
4. **Have ready:** medication list, allergies, care plan summary, DNR form, health card.
5. **Stay** until paramedics take over, then call the supervisor.

When	Who	Action
Immediately	Staff	Call 911, then supervisor
Within 1 hour	Supervisor	Notify family or SDM
Before end of shift	Staff	Incident report

Documentation: times, what happened, actions taken, and the hospital the client went to.

SOP 34: Client found unresponsive or unexpected death

Staff who find a client unresponsive call 911 immediately, and if death is obvious, leave the scene untouched.

Item	Detail
Applies to	All staff

Item	Detail
Covers	Clients found unresponsive; deaths that were not expected
Related SOPs	10 DNR, 30 Expected death, 33 Emergencies
Version / approval	1.0 · Taegan, RN

Core principles

- **When in doubt, start CPR** unless there is a valid DNR form.
- **Leave the scene as found.** Police and the coroner may need to see it.

Procedure

1. **Check and call.** Check responsiveness and breathing; call 911 immediately.
2. **CPR** unless there is a valid DNR Confirmation Form or obvious signs of death.
3. **If death is obvious,** follow 911's instructions. Do not move the client, clean up, or remove anything.
4. **Call the supervisor,** who notifies the RN and family (or supports police to do so).
5. **Support staff.** Offer a debrief.

Documentation: time found, condition, actions, and who arrived.

SOP 35: Fire and home emergencies

Staff get the client and themselves out safely and call 911 from outside.

Item	Detail
Applies to	All staff
Covers	Fire, smoke, carbon monoxide alarms, gas leaks, home hazards
Related SOPs	2 Assessment (home safety)
Version / approval	1.0 · Taegan, RN

Core principles

- **Plan the exit before the emergency,** especially for clients who cannot walk.

- **Never go back in.**

Procedure

1. **Plan at intake.** Check smoke and CO alarms work; plan the exit route; record it in the care plan.
2. **Fire, smoke or CO alarm.** Get the client out if you can do so safely, close doors behind you, call 911 from outside.
3. **If you cannot get the client out,** tell firefighters exactly where they are.
4. **Gas smell.** No switches or phones inside; leave and call from outside.
5. **Report hazards.** Blocked exits, hoarding, overloaded cords, oxygen near flames or smoking.

Documentation: incident report and any home hazards.

SOP 36: Missing client

If an at-risk client is missing and not found within 15 minutes, staff call 911.

Item	Detail
Applies to	All staff
Covers	Clients not home when expected, or who leave during a visit
Related SOPs	29 Dementia
Version / approval	1.0 · Taegan, RN

Core principles

- **Time matters,** especially in cold or hot weather and for clients with dementia.

Procedure

1. **Search close by.** Check the home and yard; call the client's phone and family.
2. **Call 911** if the client is at risk and not found within 15 minutes.
3. **Help police.** Recent photo (kept in the file with consent), description, clothing, and usual places.
4. **Call the supervisor;** stay at the home if police ask you to.
5. **Prevent.** Consider a MedicAlert or locating device in the care plan.

Documentation: timeline of events and actions.

SOP 37: Working alone and personal safety

The office always knows where staff are, and any staff member who feels unsafe leaves.

Item	Detail
Applies to	All staff, office and on-call
Covers	Client homes and community visits
Related SOPs	42 Workplace violence
Version / approval	1.0 · Taegan, RN

Core principles

- **Trust your instincts.**
- **No visit is worth your safety.**

Procedure

1. **Check in and out** with the scheduling app or by phone.
2. **Missed check-out.** After 30 minutes the office calls the staff member, then the client, then police if they cannot be reached.
3. **If unsafe** (aggression, intoxication, weapons, dangerous animals), leave and call the supervisor; call 911 if in danger.
4. **Be prepared.** Park facing out, phone charged, valuables out of sight.
5. **Report hazards** so the risk is assessed before the next visit.

Documentation: incident report for any threat or unsafe situation.

SOP 38: Severe weather and business continuity

During storms, outages and staffing shortages, high-priority clients are covered first.

Item	Detail
Applies to	Office, on-call, all staff
Covers	Severe weather, power outages, staff shortages, system outages

Item	Detail
Related SOPs	5 Scheduling
Version / approval	1.0 · Taegan, RN

Core principles

- **Know who can't be missed** before the storm hits.
- **Staff safety on the roads comes first.**

Procedure

1. **Set priorities.** Each client has a level in their file: high (e.g. insulin, lives alone), medium, or low.
2. **Cover high priority first.** The office contacts staff and clients.
3. **Unsafe roads.** Staff call the office to rearrange; they do not drive.
4. **Power outages.** Backup plan in the care plan for clients on oxygen concentrators, air mattresses, or lifts.
5. **Records.** Backed up and accessible remotely.

Documentation: visits changed or missed and why.

Section 11: Documentation and reporting

SOP 46: Clinical documentation

Every visit is documented accurately before the end of the shift, meeting CNO documentation standards.

Item	Detail
Applies to	All care staff
Covers	Visit notes, assessments, medication records, flow sheets
Related SOPs	9 Privacy, 47 Incidents, 50 Record retention
Version / approval	1.0 · Taegan, RN

Core principles

- **If it isn't documented, it wasn't done.**
- **Facts, not opinions.**

Procedure

1. **Document on time.** During or right after the visit, before the end of the shift.
2. **Write facts.** What you saw, heard, did, and the client's response; the client's own words in quotes when relevant.
3. **Identify the entry.** Approved abbreviations only; date, time, your name and designation.
4. **Never pre-chart** or document for someone else.
5. **Correct properly.** Use a late entry or correction feature (or a single line through on paper, initialled and dated). Never delete or white out.

Documentation: in the approved electronic record or client chart only.

SOP 47: Incident reporting and investigation

Every incident and near miss is reported the same shift and reviewed within 1 business day.

Item	Detail
Applies to	All staff; RN and supervisor review
Covers	Falls, medication errors, injuries, skin tears, privacy breaches, abuse concerns, aggression, missed visits, equipment failures, complaints
Related SOPs	19 Medication errors, 31 Falls, 49 Quality
Version / approval	1.0 · Taegan, RN

Core principles

- **Safe first, report second.**
- **Learn, don't blame.** Lessons are shared without naming individuals.

Procedure

1. **Make it safe,** then call the supervisor.
2. **Report** incidents and near misses.

3. **Review.** The RN or supervisor contacts the client or SDM and decides on actions.
4. **Root cause review** for serious incidents.

When	Who	Action
Immediately	Staff	Make safe, call supervisor
Before end of shift	Staff	Incident report
Within 1 business day	RN or supervisor	Review and follow-up
Monthly	Management	Trend review (SOP 49)

Documentation: incident report and review in the incident log, kept separate from the client chart; note in the chart what happened to the client.

SOP 48: Keys and home access

Keys and lockbox codes are held only with written consent and tracked in a register.

Item	Detail
Applies to	Office and care staff
Covers	Client keys, lockbox codes, door codes
Related SOPs	6 Discharge, 9 Privacy
Version / approval	1.0 · Taegan, RN

Core principles

- **A key never identifies the home.**
- **Lost keys are reported immediately.**

Procedure

1. **Get consent.** Written consent from the client or SDM.
2. **Code the keys.** Tagged with a code, never the client's name or address.
3. **Store securely.** Codes and key locations in the secure client file, shared only with assigned staff.
4. **Announce yourself.** Knock and announce on arrival even with a key; lock the door when leaving.

5. **Lost keys.** Report immediately; the office tells the client and arranges lock or code changes.

Documentation: key and code register, with sign-out and return.

SOP 51: Air travel with clients on a Pilatus PC-12

Clients who use wheelchairs or walkers are loaded onto a Pilatus PC-12 with a lifting device and cared for in flight under the pilot's authority, with every medical and equipment detail approved by the operator in advance.

Item	Detail
Applies to	Atria staff accompanying clients on PC-12 flights
Covers	Booking, boarding, lift loading, in-flight care, unloading
Does not cover	Operating the aircraft; operating the lift unless the operator has trained and authorized Atria staff
Related SOPs	32 Transfers, 33 Emergencies, 52 Vacation travel
Version / approval	1.0 · Taegan, RN

Core principles

- **The pilot-in-command has final authority** over everything on and around the aircraft. Atria staff handle the client; the operator's crew handle the aircraft and the lift.
- **Nothing approaches a turning propeller.** Loading starts only after the pilot confirms the engine is off and the propeller has stopped.
- **Approved in advance, or it doesn't fly.** Oxygen, batteries and medical devices need written operator approval.
- **Clients never fly seated in their wheelchair.**

A. Before booking (at least 7 days ahead where possible)

1. **Fitness to fly.** The RN assesses the client and obtains written medical clearance from the client's physician or NP. Flag heart or lung disease, recent surgery, recent stroke or heart attack, anemia, and any air in body spaces (e.g. recent pneumothorax or eye surgery).

2. **Cabin altitude.** The PC-12 is pressurized, but cabin altitude at cruise can be several thousand feet (up to about 10,000 ft at maximum cruise altitude). Ask the physician whether the client needs oxygen in flight, and ask the operator about flying lower if needed.
3. **Tell the operator in writing:** the client's mobility (wheelchair, walker, transfer method), weight, and height; the weight and dimensions of the wheelchair, walker, and all equipment; medical devices on board; and the number of Atria staff travelling.
4. **Dangerous goods.** Get the operator's written approval in advance for oxygen cylinders, portable oxygen concentrators, power-wheelchair or scooter batteries (especially lithium), and spare batteries. Do not bring any of these unapproved.
5. **Lifting device.** Confirm with the operator which device is used: the model, its safe working load, who is trained to operate it, and whether loading is through the forward airstair door or the rear cargo door. Confirm the client's combined weight (with chair, if lifted in it) is within the safe working load.
6. **Accessibility at both ends.** Confirm ground transport that takes the client's wheelchair, accessible FBO (terminal), accessible washroom at both FBOs, and level ground for the lift.
7. **Toileting.** Confirm whether the aircraft has a usable toilet. Plan toileting before departure and continence products for the flight.
8. **Crossing borders.** Passports, customs requirements, and medication documents (SOP 52). Atria nurses may only practise nursing where they are registered.

B. Day of travel: before boarding

1. Arrive at the FBO early so the client is not rushed. Toilet the client before boarding.
2. Give the pilot a brief: the client's mobility, how they transfer, any medical risks, and where the medication bag is.
3. Ask the pilot for the safety briefing for the client and Atria staff, including exits, seat belts, and oxygen masks if fitted. Ask how the client would be evacuated in an emergency.
4. **Never approach the aircraft until the pilot confirms the engine is shut down and the propeller has stopped.** Keep clear of the propeller at all times.
5. Check the ground: no ice, standing water, fuel, or slope at the lift position. Postpone loading in high wind, lightning, or freezing rain.

C. Loading with the lifting device

1. Only staff trained on that specific lift take part. The lift operator is in charge of the lift; one Atria staff member is in charge of the client.
2. Inspect the lift before use: platform and guard rails, safety belt, controls, emergency lowering, battery or hydraulics. If anything is faulty, do not use it.
3. Position the client on the platform facing the direction set by the operator; apply wheelchair brakes; fasten the lift's safety belt or restraint; raise the platform's roll-off barriers.
4. For a walker user: the client is seated in a wheelchair or boarding chair for the lift. Clients never stand on the lift. The walker travels as baggage.
5. One staff member stays beside the client and talks them through every movement. Raise slowly; stop if the client is distressed or anything shifts.
6. At the door, transfer the client to the aircraft seat closest to the door using the method in their care plan and a slide board or transfer belt as needed. Do not exceed manual lifting limits (SOP 32). The cabin height is limited; staff bend at the hips and knees, not the back.
7. Seat the client in an aircraft seat with the seat belt fastened. **Clients do not fly seated in their own wheelchair.**
8. Fold and secure the wheelchair and walker as the crew directs; nothing loose in the cabin. Remove and keep cushions and loose parts in the cabin with the client.

D. In flight

1. Keep the client's medication bag, continence supplies, water, and a copy of their care plan within reach in the cabin.
2. Watch for: shortness of breath, chest pain, new confusion, pale or blue lips, ear pain, nausea, or anxiety. Use a pulse oximeter if the RN has ordered it.
3. Give oxygen only as prescribed and approved by the operator.
4. Encourage fluids and ankle and calf exercises to lower clot risk; keep the client warm.
5. Stay seated with belts fastened when the pilot directs. Tell the pilot immediately about any medical change; the pilot decides on diversion.

E. Unloading and arrival

1. Wait for the pilot to confirm the engine is shut down and the propeller has stopped.

2. Reverse the loading steps: wheelchair checked and set up at the door, transfer, secure on the lift, lower slowly.
3. Check the client for pressure areas, swelling, and how they are feeling before ground transport.

Document: pre-flight assessment and clearance, operator approvals, the lift used and who operated it, in-flight observations and care, and any incident.

SOP 52: Travelling with clients on vacation

Travel with a client starts with an RN travel assessment and a written travel care plan at least 6 weeks before departure, and staff follow Atria's SOPs throughout the trip.

Item	Detail
Applies to	RN (plans), all Atria staff travelling with clients
Covers	Vacations and trips in Canada and abroad, by any transport
Related SOPs	18–20 Medications, 31 Falls, 33 Emergencies, 51 PC-12 air travel
Version / approval	1.0 · Taegan, RN

Core principles

- **Plan early.** Insurance, accessibility, equipment and medication rules take weeks to sort out.
- **Scope doesn't travel.** Atria nurses may only practise nursing where they are registered.
- **Staff are working, not on holiday.** Rest periods and hours are agreed in writing.

A. Planning (start at least 6 weeks before departure)

1. **Travel assessment.** The RN assesses whether the trip is safe and what care is needed, and gets written medical clearance from the client's physician or NP for air travel or long trips.
2. **Travel care plan.** The RN writes a trip-specific care plan: daily care schedule, transfers, medications, risks, emergency plan, and how Atria supports staff remotely.

3. **Staffing.** Decide the number and type of staff needed for the trip.
4. **Scope across borders.** Atria nurses may only practise nursing where they are registered. Outside Ontario (another province or a US state), check registration rules before travel. If staff are not registered there, they travel in a companion or personal support role only, and the travel care plan says so.
5. **Insurance.** The client buys out-of-province or out-of-country medical insurance that covers pre-existing conditions and medical evacuation; OHIP covers very little outside Canada. Confirm Atria's liability insurance covers staff at the destination.
6. **Accessibility.** Confirm accessible accommodation (roll-in shower, bed height, door widths, grab bars), accessible transport at the destination, and rental of equipment there (e.g. hospital bed, commode, lift, oxygen).
7. **Medications abroad.** Check the destination country's rules for the client's medications; some countries restrict opioids, stimulants, cannabis and sleeping pills. Get a physician's letter listing all medications and diagnoses.
8. **Emergency plan.** Write down the local emergency number, the nearest hospital with an emergency department, a local pharmacy, the insurer's 24-hour line, the Canadian embassy or consulate (abroad), and Atria's on-call number.
9. **Consent.** The client or SDM signs the travel care plan and consents to sharing health information with providers at the destination if needed.

B. During the trip

1. Follow the travel care plan and the client's usual routines as closely as possible.
2. Check in with the Atria RN or on-call at least daily, and right away for any change in condition, fall, or incident.
3. Use the same SOPs as at home: falls (SOP 31), transfers (SOP 32), medications (SOP 18 to 20), emergencies (SOP 33).
4. Adjust for heat, cold, altitude, time zones, and unfamiliar food and water; watch for dehydration, constipation, and confusion.
5. Check each new room or space for hazards: rugs, bathroom floors, bed height, lighting at night.
6. Respect boundaries: staff are working, not on holiday. No alcohol on shift; personal time only during scheduled time off.

C. Returning home

1. The RN checks the client within 48 hours of return.

2. Reconcile medications, and update the care plan with any changes.
3. Staff complete documentation and expense reports within 5 business days.

Document: travel assessment and clearance, travel care plan, daily care notes, daily check-ins, and any incident reports.

Preparation checklist

Documents

- [ ] Passport or ID for client and staff (valid at least 6 months past return for travel abroad)
- [ ] Ontario health card and travel medical insurance card with the 24-hour assistance number
- [ ] Physician's letter listing diagnoses and all medications
- [ ] Travel care plan, medication list, allergy list, and emergency contacts (paper copy plus a secure electronic copy)
- [ ] DNR Confirmation Form and advance directive, if applicable
- [ ] Booking confirmations: flights, accommodation, accessible transport, equipment rentals
- [ ] Operator approvals for oxygen, batteries and medical devices

Medications

- [ ] All medications in original pharmacy-labelled containers or blister packs
- [ ] At least 7 days' extra supply beyond the trip length
- [ ] Carried in hand luggage, split between two bags if possible
- [ ] Cooler bag and ice packs for medications needing refrigeration (e.g. insulin)
- [ ] Plan for time zone changes, written by the RN
- [ ] Epinephrine auto-injectors, glucose tablets, and inhalers within reach

Mobility and equipment

- [ ] Wheelchair or walker checked: brakes, tires, tips, folding mechanism; name and contact tag attached
- [ ] Wheelchair cushion and removable parts kept with the client
- [ ] Transfer belt and slide board or slide sheet
- [ ] Portable raised toilet seat or commode if not available at the destination
- [ ] Oxygen or portable oxygen concentrator, charger and spare batteries (if approved)
- [ ] Pressure-relieving cushion for long trips

Care supplies

- [ ] Continence products for the trip plus 3 extra days, wipes, barrier cream, disposal bags
- [ ] Wound care and dressing supplies, as ordered
- [ ] Blood glucose meter, strips, lancets, sharps container
- [ ] BP monitor, thermometer, pulse oximeter (as ordered)
- [ ] Gloves, hand sanitizer, masks, disinfectant wipes
- [ ] First aid kit

Comfort and safety

- [ ] Comfortable, non-slip shoes and layers for changing temperatures
- [ ] Glasses, hearing aids with spare batteries, dentures and their case
- [ ] Personal emergency response or fall-detection device, with coverage confirmed at the destination
- [ ] Night light, and familiar items to reduce confusion (photos, pillow)
- [ ] Water bottle and healthy snacks for travel days
- [ ] Phone, chargers, and power adapter for the destination

Staff

- [ ] Signed travel work agreement: hours, rest periods, pay and expenses
- [ ] Atria ID badge, professional registration proof
- [ ] Atria on-call number and RN contact saved
- [ ] Personal medications and travel insurance